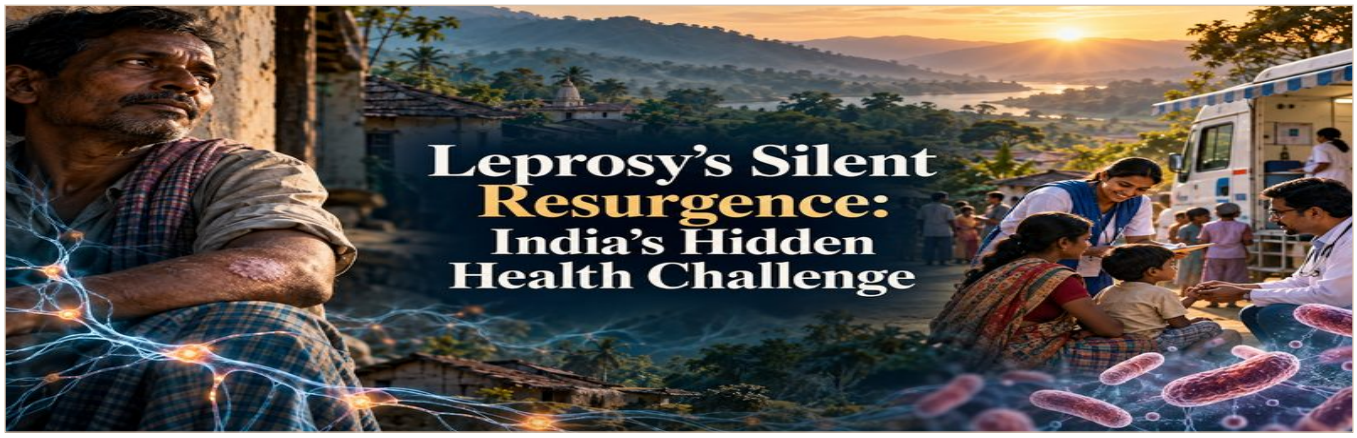


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[ GS PAPER I – SOCIETY • GS PAPER II – GOVERNANCE, SOCIAL JUSTICE & HEALTH • GS PAPER III – SCIENCE & TECHNOLOGY ]
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# Leprosy's Silent Resurgence: India's Hidden Health Challenge




*India's leprosy elimination milestone masks persistent disease burdens, undetected cases and healthcare gaps, particularly in Chhattisgarh. The article highlights the need for early diagnosis, effective treatment and a rights-based approach that addresses stigma and protects dignity.*

## Leprosy's Silent Resurgence: India's Hidden Health Challenge

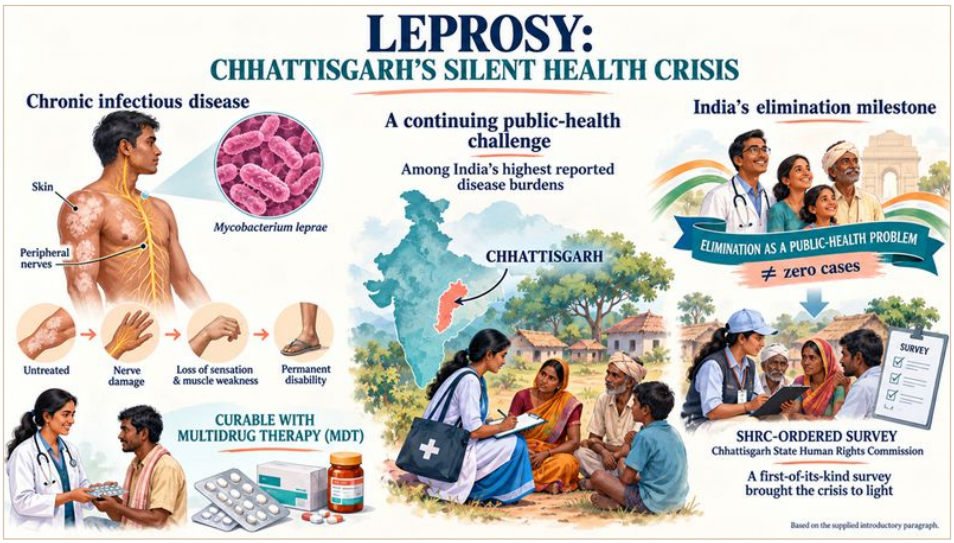
## Syllabus areas:

GS Paper I – Society

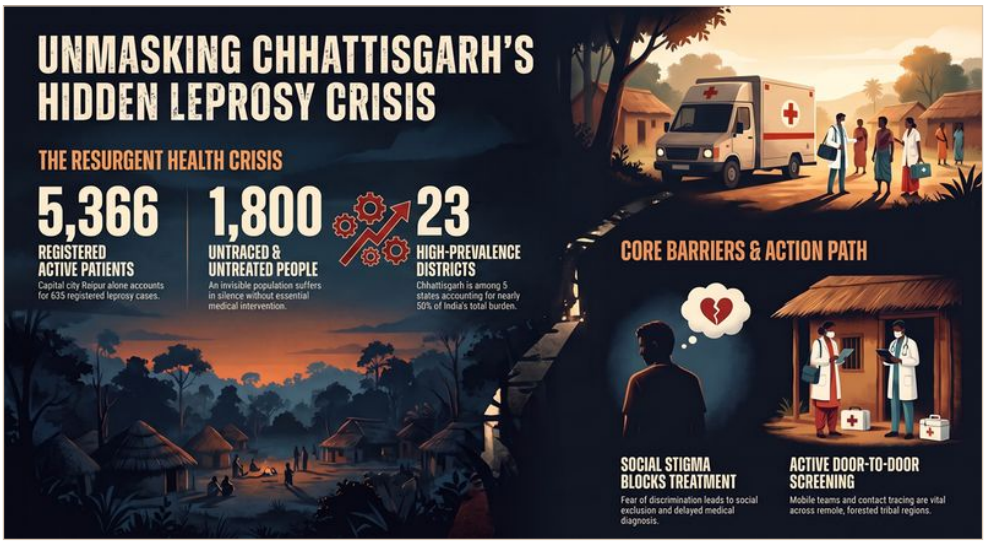
GS Paper II – Governance, Social Justice & Health

GS Paper III – Science & Technology

**Leprosy** is a **chronic infectious disease caused by *Mycobacterium leprae***, primarily affecting the skin and peripheral nerves. If untreated, it can cause nerve damage, loss of sensation, muscle weakness and permanent disabilities. It is **curable with multidrug therapy (MDT)**.




WHILE India celebrated the elimination of leprosy as a public health milestone nearly two decades ago, a silent, raging health crisis is unfolding in **Chhattisgarh**, which is battling one of the highest rates of the disease spread in the country.




"The revelation came to light following a first-of-its-kind survey ordered by the Chhattisgarh State Human Rights Commission (SHRC).

**State Human Rights Commission (SHRC)** is a statutory body established under the **Protection of Human Rights Act, 1993** to protect and promote human rights at the State level. It can inquire into human-rights violations, recommend remedial measures, and promote awareness of human-rights protections.

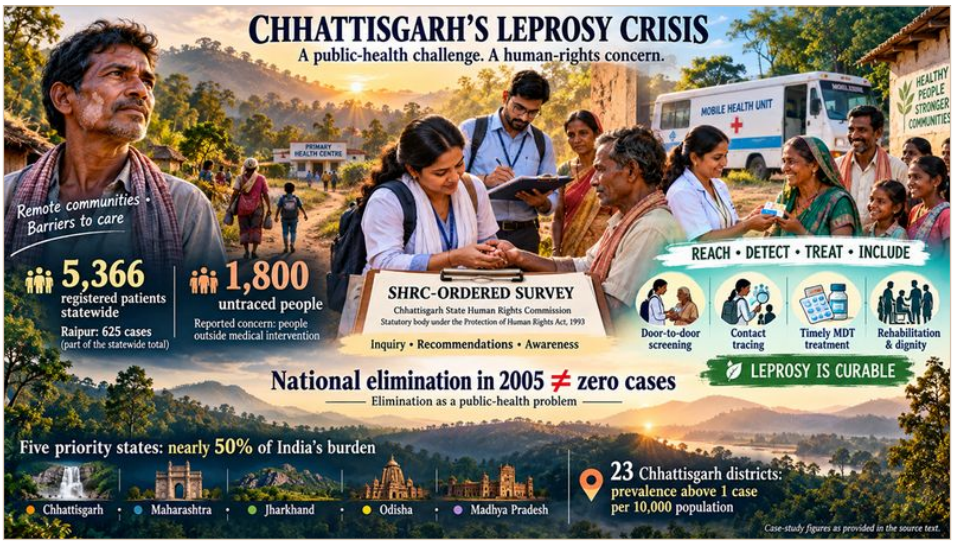
Social Stigma & Human Rights in Leprosy




Leprosy is medically curable, yet **stigma, discrimination and social exclusion** can discourage affected persons from seeking diagnosis and treatment. This creates both health and human-rights concerns. A rights-based approach requires **dignity, non-discrimination, rehabilitation, awareness and social inclusion**, alongside effective healthcare. Institutions such as Human Rights Commissions can strengthen accountability and protection.

Chhattisgarh continues to report cases of leprosy, a communicable bacterial infection which, though not aggressively contagious, can lead to severe nerve damage and physical disabilities if left untreated.

Good health is a part of human rights.




According to official records made available to the commission, there are currently 5,366 registered leprosy patients across the state, with capital Raipur recording 625 cases. However, the invisible number of 1,800 untraced people—suffering in silence without medical intervention—has triggered concerns.

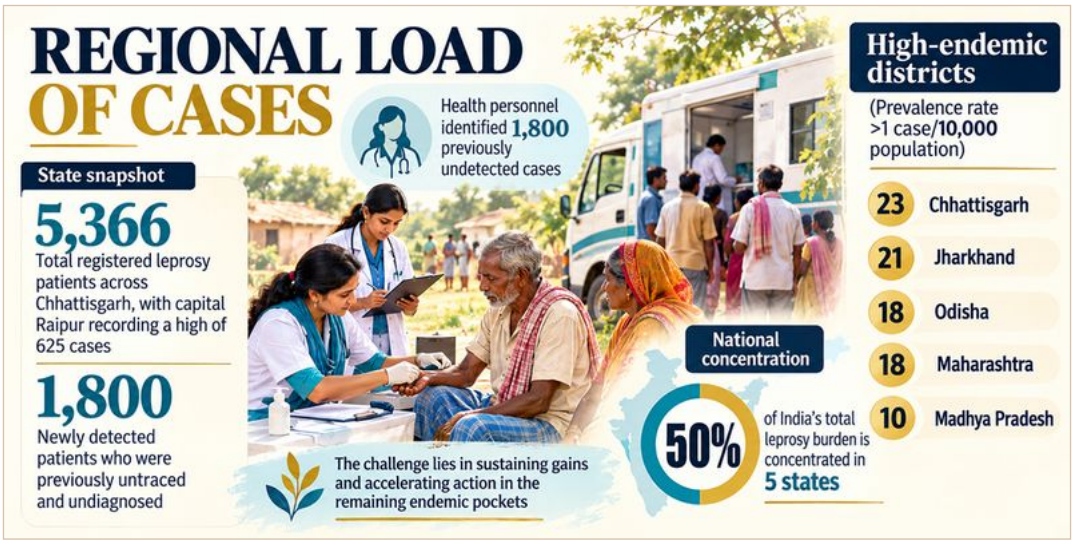
India attained elimination of the burden of leprosy as a public health problem at the national level in 2005. Five high-priority states—Chhattisgarh, Maharashtra, Jharkhand, Odisha, and Madhya Pradesh—together account for nearly 50% of India's leprosy burden. These states also have a substantial number of districts reporting prevalence rates above one case per 10,000 population, including 23 districts in Chhattisgarh, 21 in Jharkhand, 18 each in Maharashtra and Odisha, and 10 districts in Madhya Pradesh.

Tribal Health & Healthcare Accessibility in Chhattisgarh

Chhattisgarh has a significant tribal population living across **forested, hilly and geographically remote regions**, creating challenges in healthcare access. Leprosy control requires **active case detection, door-to-door screening, contact tracing and timely treatment**. Strengthening primary healthcare, mobile medical teams, community participation and culturally sensitive outreach can improve early diagnosis and prevent disability.

One Health Approach & Integrated Disease Surveillance

An integrated disease-control approach connects **early diagnosis, contact tracing, active surveillance, preventive treatment and community participation** with primary healthcare. Strengthening surveillance systems enables identification of hidden cases and endemic pockets, while coordinated health interventions improve treatment adherence, prevent transmission and reduce disability, supporting India's broader communicable-disease elimination goals.




For more details read article: **Leprosy in India: The Elimination Gap**